



Bill Number: S.B. 1611

Werner Floor Amendment

**Reference to: HEALTH AND HUMAN SERVICES
Committee Amendment**

Amendment drafted by: Leg. Council

FLOOR AMENDMENT EXPLANATION

1. Excludes services provided to an American Indian or Alaska Native (AI/AN) through a tribal health program from the oversight of the administrative services organization.
2. Requires the Arizona Health Care Cost Containment System (AHCCCS), as part of any request for proposals for an American Indian Health Program (AIHP) administrative services organization, to require each offeror to demonstrate a proven history of competency and experience in providing robust case management and complaint services to AI/AN populations in coordination with tribal health programs.
3. Requires AHCCCS, before issuing any request for proposals for an administrative services organization, to complete a meaningful and timely tribal consultation and urban confer regarding implementation and subsequently, within 30 days, submit any necessary state plan amendment or waiver to the U.S. Centers for Medicare and Medicaid Services.
4. Replaces legislative appointments of observers to the AHCCCS source-selection committee with appointments by each federally recognized Indian Tribe in Arizona and requires each observer to be a member of a different tribe.
5. Requires the administrative services organization to establish a streamlined provider screening and registration process to ensure only qualified providers are enrolled and prohibits the administrative services organization from requiring any additional registration.
6. Prohibits the administrative services organization from requiring prior authorization for services provided to an AIHP member when the member is referred from a tribal health program to a nontribal provider or facility.
7. Requires the administrative services organization to conduct its duties in an efficient, competent and culturally informed manner and prohibits the administrative services organization from interfering with or overriding the clinical determinations or care management decisions of a referring tribal health program.
8. Requires the administrative services organization to establish an office of tribal relations and outlines duties of the office.
9. Requires AHCCCS and the administrative services organization to consult quarterly with the Governor, tribal health programs, federally recognized Indian Tribes, AHCCCS providers and AIHP members to evaluate the administrative services organization's performance and data analytics to ensure tribal members are receiving quality, safe and appropriate care.
10. Modifies defined terms.

WERNER FLOOR AMENDMENT
SENATE AMENDMENTS TO S.B. 1611
(Reference to HEALTH AND HUMAN SERVICES Committee amendment)

Amendment instruction key:

[GREEN UPPERCASE UNDERLINING IN BRACKETS] indicates that the amendment is adding text to statute or previously enacted session law.
[Green lowercase underlining in brackets] indicates that the amendment is adding text to new session law or is restoring previously stricken text to existing statute.
~~[GREEN UPPERCASE STRIKEOUT IN BRACKETS]~~ indicates that the amendment is removing new text from statute or previously enacted session law.
~~[Green lowercase strikeout in brackets]~~ indicates that the amendment is removing text from existing statute, previously enacted session law or new session law.
<<Double green carets enclosing an entire section>> indicates that the amendment is adding the section to the bill.
~~<<Green strikeout with double green carets enclosing an entire section>>~~ indicates that the amendment is removing the section to the bill.
{[ORANGE UPPERCASE UNDERLINING IN DOUBLE CURLY BRACKETS]} indicates that the amendment to an amendment is adding text to statute or previously enacted session law.
{[Orange lowercase underlining in double curly brackets]} indicates that the amendment to an amendment is adding text to new session law or is restoring previously stricken text to existing statute.
~~{[ORANGE UPPERCASE STRIKEOUT IN DOUBLE CURLY BRACKETS]}~~ indicates that the amendment to an amendment is removing new text from statute or previously enacted session law.
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<<Double orange underlined carets enclosing an entire section>> indicate that the amendment to an amendment is adding the section to the bill.
~~<<Orange strikeout with double orange underlined carets enclosing an entire section>>~~ indicates that the amendment to an amendment is removing the section from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 36, chapter 29, article 1, Arizona Revised
3 Statutes, is amended by adding section 36-2903.18, to read:

4 36-2903.18. American Indian health program; administrative
5 services contract; legislative review;
6 administrative services organization; duties;
7 office of tribal relations; definitions

8 A. BEGINNING OCTOBER 1, 2027, THE ADMINISTRATION SHALL CONTRACT
9 WITH A QUALIFIED CONTRACTOR OR OTHER QUALIFIED ENTITY TO SERVE AS THE
10 ADMINISTRATIVE SERVICES ORGANIZATION AND TO PERFORM ~~[PROGRAM INTEGRITY AND~~
11 ~~CARE MANAGEMENT]~~ [THE] FUNCTIONS [PRESCRIBED IN SUBSECTION {{H-}} {{I}} OF
12 THIS SECTION] FOR MEMBERS ENROLLED IN THE AMERICAN INDIAN HEALTH PROGRAM~~],~~
13 ~~NOT INCLUDING SERVICES PROVIDED TO AMERICAN INDIAN OR ALASKA NATIVE~~
14 ~~MEMBERS THROUGH {{THE INDIAN HEALTH SERVICE OR}} A TRIBAL {{FACILITY}}~~
15 ~~{{HEALTH PROGRAM}}].~~ THE CONTRACT MUST COMPLY WITH ALL APPLICABLE FEDERAL
16 AND STATE REQUIREMENTS, INCLUDING THE PROTECTIONS FOR INDIANS, INDIAN

1 HEALTH CARE PROVIDERS AND INDIAN MANAGED CARE ENTITIES PURSUANT TO 42 CODE
2 OF FEDERAL REGULATIONS SECTION 438.14.

3 B. THE ADMINISTRATION SHALL RETAIN ULTIMATE RESPONSIBILITY FOR
4 ADMINISTERING THE AMERICAN INDIAN HEALTH PROGRAM, INCLUDING FEE-FOR-SERVICE
5 RATE SETTING[, ~~CLAIMS PAYMENT AUTHORITY~~] AND OVERSIGHT OF ANY
6 ADMINISTRATIVE SERVICES ORGANIZATION CONTRACTOR, AND MAY NOT ELIMINATE THE
7 AMERICAN INDIAN HEALTH PROGRAM AS A FEE-FOR-SERVICE OPTION FOR ELIGIBLE
8 AMERICAN INDIAN AND ALASKA NATIVE MEMBERS.

9 ~~{{C. AS PART OF ANY REQUEST FOR PROPOSALS ISSUED BY THE
10 ADMINISTRATION FOR AN ADMINISTRATIVE SERVICES ORGANIZATION PURSUANT TO
11 THIS SECTION. THE ADMINISTRATION SHALL REQUIRE EACH OFFEROR TO DEMONSTRATE
12 A PROVEN HISTORY OF COMPETENCY AND EXPERIENCE IN PROVIDING ROBUST CASE
13 MANAGEMENT AND COMPASSIONATE, QUALITY AND COMPLIANT SERVICES TO AMERICAN
14 INDIAN AND ALASKA NATIVE POPULATIONS AND IN COORDINATING WITH TRIBAL
15 HEALTH PROGRAMS.}}~~

16 ~~{{C.}}~~ ~~{{D.}}~~ BEFORE ISSUING ANY REQUEST FOR PROPOSALS TO PROCURE
17 AN ADMINISTRATIVE SERVICES ORGANIZATION PURSUANT TO SUBSECTION A OF THIS
18 SECTION:

19 1. ~~{{NOT LATER THAN THIRTY DAYS AFTER THE EFFECTIVE DATE OF THIS
20 SECTION, THE DIRECTOR SHALL REQUEST ANY NECESSARY APPROVALS FROM}} {{THE
21 ADMINISTRATION SHALL COMPLETE MEANINGFUL AND TIMELY TRIBAL CONSULTATION
22 AND URBAN CONFER REGARDING THE IMPLEMENTATION OF THIS SECTION AND, WITHIN
23 THIRTY DAYS AFTER COMPLETING THE TRIBAL CONSULTATION AND URBAN CONFER,
24 SHALL SUBMIT ANY STATE PLAN AMENDMENT OR WAIVER TO}} THE CENTERS FOR
25 MEDICARE AND MEDICAID SERVICES {{TO IMPLEMENT THIS SECTION AND TO QUALIFY
26 FOR FEDERAL MONIES AVAILABLE UNDER TITLE XIX OF THE SOCIAL SECURITY ACT OR
27 THE SECTION 1115 WAIVER}}.~~

28 2. NOTWITHSTANDING ANY LAW TO THE CONTRARY:

29 (a) THE ADMINISTRATION SHALL ~~{{PRESENT}}~~ ~~{{TRANSMIT}}~~ THE PROPOSED
30 PROCUREMENT STRATEGY, MINIMUM QUALIFICATIONS AND EVALUATION FACTORS TO THE
31 SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN SERVICES COMMITTEES,
32 OR THEIR SUCCESSOR COMMITTEES.

33 (b) THE ADMINISTRATION SHALL RECEIVE~~{{.}}~~ ~~{{AND}}~~ ~~[THOUGHTFULLY]~~
34 ~~CONSIDER [AND {{MAY}} {{MEANINGFULLY AND TIMELY}} RESPOND TO]~~ WRITTEN AND
35 ORAL COMMENTS FROM COMMITTEE MEMBERS AND THE PUBLIC, INCLUDING TRIBAL
36 GOVERNMENTS, INDIAN HEALTH CARE PROVIDERS AND URBAN INDIAN ORGANIZATIONS.

37 ~~{{D.}}~~ ~~{{E.}}~~ NOTWITHSTANDING ANY LAW TO THE CONTRARY, FOR EACH
38 PROCUREMENT DESCRIBED IN SUBSECTION ~~{{C.}}~~ ~~{{D}}~~ OF THIS SECTION:

39 1. ~~{{THE}}~~ ~~[CHAIRPERSONS]~~ ~~{{[PRESIDENT] OF THE SENATE AND [THE
40 SPEAKER OF THE] HOUSE OF REPRESENTATIVES}}~~ ~~[HEALTH AND HUMAN SERVICES
41 COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, MAY EACH DESIGNATE ONE MEMBER
42 OF THEIR RESPECTIVE COMMITTEE TO SERVE AS A] {{[SHALL EACH APPOINT ELEVEN]
43 NONVOTING}} [OBSERVER] {{[OBSERVERS WHO ARE MEMBERS OF DIFFERENT FEDERALLY
44 RECOGNIZED INDIAN TRIBES]}}~~ ~~{{EACH FEDERALLY RECOGNIZED INDIAN TRIBE IN
45 THIS STATE MAY APPOINT ONE NONVOTING OBSERVER}}~~ TO THE SOURCE-SELECTION
46 EVALUATION COMMITTEE CONVENED BY THE ADMINISTRATION. ~~{{EACH OBSERVER MUST
47 BE A MEMBER OF A DIFFERENT FEDERALLY RECOGNIZED INDIAN TRIBE IN THIS
48 STATE.}}~~

1 2. THE ~~{}TWENTY-TWO{}~~ NONVOTING ~~{}TRIBAL MEMBER{}~~ OBSERVERS
2 DESIGNATED PURSUANT TO PARAGRAPH 1 OF THIS SUBSECTION MAY ATTEND
3 EVALUATION MEETINGS~~{}{.}{}~~ ~~{}AND{}~~ REVIEW MATERIALS SUBMITTED BY OFFERORS~~{}{.}~~
4 ASK QUESTIONS AND MAKE STATEMENTS DURING THE EVALUATION MEETINGS BUT MAY
5 NOT SCORE PROPOSALS, PARTICIPATE IN FINAL RANKING OR MAKE OR DIRECT AN
6 AWARD DECISION.

7 ~~{}F-{}{}~~ ~~{}F-{}{}~~ NOTWITHSTANDING ANY LAW TO THE CONTRARY, AFTER THE
8 COMPLETION OF PROPOSAL EVALUATIONS AND BEFORE THE EXECUTION OF ANY
9 CONTRACT DESCRIBED IN SUBSECTION A OF THIS SECTION, THE ADMINISTRATION
10 SHALL TRANSMIT ~~{}AND-PRESENT{}~~ TO THE SENATE AND HOUSE OF REPRESENTATIVES
11 HEALTH AND HUMAN SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES, A
12 SUMMARY OF EVALUATION RESULTS AND THE PROPOSED AWARD RECOMMENDATION.

13 ~~{}F-{}{}~~ ~~{}G-{}{}~~ THIS SECTION DOES NOT:

14 1. TRANSFER SOURCE-SELECTION OR CONTRACT-AWARD AUTHORITY FROM THE
15 ADMINISTRATION TO THE LEGISLATURE~~{}{.}~~ ~~{}OR{}~~ ANY LEGISLATIVE COMMITTEE ~~{}OR~~
16 ANY TRIBAL GOVERNMENT.

17 2. CONFLICT WITH THE ARIZONA PROCUREMENT CODE OR ANY APPLICABLE
18 FEDERAL PROCUREMENT REQUIREMENT. THE ADMINISTRATION REMAINS THE AWARDED
19 AGENCY AND SHALL COMPLY WITH ALL APPLICABLE STATE AND FEDERAL PROCUREMENT
20 LAWS AND REGULATIONS.

21 ~~{}G-{}{}~~ ~~{}H-{}{}~~ ANY ADMINISTRATIVE ARRANGEMENT AUTHORIZED UNDER THIS
22 SECTION MUST PRESERVE:

23 1. THE RIGHT OF ELIGIBLE AMERICAN INDIAN AND ALASKA NATIVE MEMBERS
24 TO ELECT THE AMERICAN INDIAN HEALTH PROGRAM FEE-FOR-SERVICE COVERAGE.

25 2. THE RIGHT OF AMERICAN INDIAN AND ALASKA NATIVE MEMBERS TO
26 VOLUNTARILY ENROLL IN AND DISENROLL FROM A MANAGED CARE ORGANIZATION
27 CONSISTENT WITH FEDERAL LAW.

28 3. THE PROTECTIONS AFFORDED TO INDIANS AND INDIAN HEALTH CARE
29 PROVIDERS UNDER 42 CODE OF FEDERAL REGULATIONS SECTION 438.14, INCLUDING
30 OUT-OF-NETWORK ACCESS AND APPROPRIATE PAYMENT REQUIREMENTS FOR INDIAN
31 HEALTH CARE PROVIDERS.

32 ~~{}H-{}{}~~ ~~{}I-{}{}~~ ANY ENTITY THAT IS PROCURED PURSUANT TO THIS SECTION
33 TO PERFORM PROGRAM INTEGRITY~~{}{.}~~ ~~{}OR{}~~ CARE MANAGEMENT~~{}{. PROVIDER SUPPORT,~~
34 QUALITY IMPROVEMENT AND DATA ANALYTICS FUNCTIONS [AND CLAIMS PAYMENT] FOR
35 THE AMERICAN INDIAN HEALTH PROGRAM SHALL BE ORGANIZED AND OPERATED AS A
36 QUALIFIED CONTRACTOR. [THE ADMINISTRATIVE SERVICES ORGANIZATION'S
37 PERFORMANCE OF PROGRAM INTEGRITY FUNCTIONS DOES NOT SUPERSEDE THE DUTIES
38 AND RESPONSIBILITIES OF THE ADMINISTRATION'S OFFICE OF INSPECTOR GENERAL
39 TO INVESTIGATE FRAUD.]

40 ~~{}J-{}{}~~ THE ADMINISTRATIVE SERVICES ORGANIZATION AND THE
41 ADMINISTRATION SHALL ESTABLISH A STREAMLINED PROVIDER SCREENING AND
42 REGISTRATION PROCESS TO ENSURE THAT ONLY QUALIFIED PROVIDERS ARE ISSUED A
43 VALID ARIZONA HEALTH CARE COST CONTAINMENT SYSTEM PROVIDER IDENTIFICATION
44 NUMBER. THE ADMINISTRATIVE SERVICES ORGANIZATION MAY NOT REQUIRE ANY
45 ADDITIONAL PROVIDER REGISTRATION. THIS SUBSECTION DOES NOT PROHIBIT THE
46 ADMINISTRATION OR THE ADMINISTRATIVE SERVICES ORGANIZATION FROM MONITORING
47 PROVIDERS TO ENSURE COMPLIANCE WITH APPLICABLE LAWS, RULES AND POLICIES.

1 K. THE ADMINISTRATIVE SERVICES ORGANIZATION IS PROHIBITED FROM
2 REQUIRING PRIOR AUTHORIZATION FOR ANY SERVICES BEING PROVIDED TO AN
3 AMERICAN INDIAN HEALTH PROGRAM MEMBER WHEN THE MEMBER IS BEING REFERRED
4 FROM A TRIBAL HEALTH PROGRAM TO A NONTRIBAL HEALTH PROVIDER OR FACILITY.
5 THE ADMINISTRATION AND THE ADMINISTRATIVE SERVICES ORGANIZATION MAY NOT
6 REQUIRE PRIOR AUTHORIZATION FOR ANY SERVICES BEING PROVIDED TO AN AMERICAN
7 INDIAN HEALTH PROGRAM MEMBER BY A NONTRIBAL HEALTH PROVIDER OR FACILITY IN
8 A MANNER THAT IS DIFFERENT FROM THE ADMINISTRATION'S POLICY IN EFFECT ON
9 JANUARY 1, 2026. ANY CHANGES TO PRIOR AUTHORIZATION REQUIREMENTS OR
10 POLICY THAT AFFECT THE AMERICAN INDIAN HEALTH PROGRAM MAY NOT BE
11 IMPLEMENTED UNTIL AFTER THE QUARTERLY TRIBAL CONSULTATION REQUIRED BY
12 SUBSECTION N OF THIS SECTION.

13 L. THE ADMINISTRATIVE SERVICES ORGANIZATION:

14 1. SHALL CONDUCT ITS DUTIES IN A MANNER THAT IS CORRECTIVE IN
15 NATURE, EFFICIENT, COMPETENT, STATE-OF-THE-ART AND CULTURALLY INFORMED.

16 2. IF AN AMERICAN INDIAN HEALTH PROGRAM MEMBER RECEIVES SERVICES
17 FROM A NONTRIBAL HEALTH PROGRAM PURSUANT TO A REFERRAL FROM A TRIBAL
18 HEALTH PROGRAM, SHALL:

19 (a) WHEN APPROPRIATE, COORDINATE CARE MANAGEMENT AND UTILIZATION
20 MANAGEMENT ACTIVITIES WITH THE TRIBAL HEALTH PROGRAMS.

21 (b) FACILITATE COMMUNICATION REGARDING REFERRALS, AUTHORIZATIONS
22 AND CLAIMS PROCESSING AND PAYMENT ISSUES.

23 3. MAY NOT INTERFERE WITH OR OVERRIDE THE CLINICAL DETERMINATIONS
24 OR CARE MANAGEMENT DECISIONS OF THE REFERRING TRIBAL HEALTH PROGRAM.

25 M. THE ADMINISTRATIVE SERVICES ORGANIZATION SHALL ESTABLISH AN
26 OFFICE OF TRIBAL RELATIONS, WHICH SHALL:

27 1. COORDINATE AND COMMUNICATE WITH THE FEDERALLY RECOGNIZED INDIAN
28 TRIBES IN THIS STATE, TRIBAL MEMBERS AND TRIBAL HEALTH PROGRAMS.

29 2. SERVE AS THE PRIMARY POINT OF CONTACT BETWEEN THE ADMINISTRATIVE
30 SERVICES ORGANIZATION AND TRIBAL GOVERNMENTS AND TRIBAL HEALTH PROGRAMS.

31 3. PARTICIPATE IN TRIBAL CONSULTATION AND STAKEHOLDER MEETINGS AS
32 REQUESTED BY TRIBAL GOVERNMENTS OR THE ADMINISTRATION.

33 N. THE ADMINISTRATION AND THE ADMINISTRATIVE SERVICES ORGANIZATION
34 SHALL CONSULT QUARTERLY WITH THE GOVERNOR OR THE GOVERNOR'S DESIGNEE,
35 TRIBAL HEALTH PROGRAMS, REPRESENTATIVES OF THE FEDERALLY RECOGNIZED INDIAN
36 TRIBES IN THIS STATE, ARIZONA HEALTH CARE COST CONTAINMENT SYSTEM
37 PROVIDERS AND AMERICAN INDIAN HEALTH PROGRAM MEMBERS TO EVALUATE THE
38 PERFORMANCE OF THE ADMINISTRATIVE SERVICES ORGANIZATION AND DATA ANALYTICS
39 GATHERED FROM THE PRECEDING QUARTER BY THE PARTIES TO ENSURE THAT TRIBAL
40 MEMBERS ARE RECEIVING QUALITY, SAFE AND APPROPRIATE CARE.}}

41 ~~{{F.}}~~ ~~{{Q.}}~~ FOR THE PURPOSES OF THIS SECTION:

42 1. "ADMINISTRATIVE SERVICES ORGANIZATION" MEANS A MEDICAID MANAGED
43 CARE ORGANIZATION THAT IS A QUALIFIED CONTRACTOR OR OTHER QUALIFIED ENTITY
44 THAT CONTRACTS WITH THE ADMINISTRATION TO PERFORM ADMINISTRATIVE
45 FUNCTIONS, INCLUDING PROGRAM INTEGRITY, CARE MANAGEMENT, PROVIDER
46 [NETWORK] SUPPORT, QUALITY IMPROVEMENT[.] [AND] DATA ANALYTICS [AND CLAIMS
47 PAYMENT].

1 2. "AMERICAN INDIAN HEALTH PROGRAM"[;:

2 (a)] MEANS THE FEE-FOR-SERVICE PROGRAM OPERATED BY THE
3 ADMINISTRATION FOR ELIGIBLE AMERICAN INDIAN AND ALASKA NATIVE MEMBERS AS
4 REQUIRED UNDER FEDERAL LAW TO PRESERVE A FEE-FOR-SERVICE OPTION FOR
5 AMERICAN INDIAN AND ALASKA NATIVE BENEFICIARIES.

6 [(b) DOES NOT INCLUDE SERVICES PROVIDED TO AMERICAN INDIAN OR
7 ALASKA NATIVE MEMBERS THROUGH ~~{{THE INDIAN HEALTH SERVICE OR}}~~ A TRIBAL
8 ~~{{FACILITY}}~~ ~~{{HEALTH PROGRAM}}~~].

9 3. "CARE MANAGEMENT" MEANS THE COORDINATION OF A MEMBER'S HEALTH
10 CARE SERVICES TO ENSURE MEDICALLY NECESSARY AND APPROPRIATE CARE.

11 4. "DATA ANALYTICS" MEANS THE COLLECTION, INTEGRATION, ANALYSIS AND
12 REPORTING OF PROGRAM CLAIMS AND PROVIDER AND MEMBER DATA TO SUPPORT
13 PROGRAM OPERATIONS, DECISION-MAKING AND OVERSIGHT.

14 5. "PROGRAM INTEGRITY" MEANS FUNCTIONS DESIGNED TO PREVENT, DETECT
15 AND INVESTIGATE FRAUD, WASTE OR ABUSE AND TO RECOVER IMPROPER PAYMENTS IN
16 THE ADMINISTRATION OR DELIVERY OF COVERED SERVICES.

17 6. "PROVIDER SUPPORT" INCLUDES:

18 (a) PROVIDER EDUCATION, TRAINING AND TECHNICAL ASSISTANCE.

19 (b) ASSISTANCE WITH CLAIMS SUBMISSION AND BILLING ISSUES.

20 (c) COMMUNICATION OF PROGRAM POLICIES, PROCEDURES AND UPDATES.]

21 ~~[5:]~~ [7.] "QUALIFIED CONTRACTOR" OR "OTHER QUALIFIED ENTITY" MEANS
22 AN ENTITY THAT MEETS ALL APPLICABLE FEDERAL AND STATE REQUIREMENTS TO
23 CONTRACT WITH THE ADMINISTRATION AS A MEDICAID MANAGED CARE ORGANIZATION,
24 ADMINISTRATIVE SERVICES ORGANIZATION OR SIMILAR ENTITY.

25 [8. "QUALITY IMPROVEMENT" MEANS MEASURING, MONITORING AND IMPROVING
26 THE QUALITY, SAFETY AND OUTCOMES OF HEALTH CARE SERVICES.]

27 ~~{{9. "TRIBAL HEALTH PROGRAM" MEANS ANY OF THE FOLLOWING:~~

28 (a) A TRIBAL FACILITY THAT IS OPERATED BY AN INDIAN TRIBE OR TRIBAL
29 ORGANIZATION AND THAT IS AUTHORIZED TO PROVIDE SERVICES PURSUANT TO PUBLIC
30 LAW 93-638, AS AMENDED.

31 (b) ANY HEALTH CARE PROVIDER OR FACILITY THAT IS OPERATED BY THE
32 INDIAN HEALTH SERVICE.

33 (c) A TRIBAL HEALTH PROGRAM OR FACILITY THAT IS OPERATED BY A
34 FEDERALLY RECOGNIZED INDIAN TRIBE IN THIS STATE.

35 (d) ANY HEALTH CARE PROVIDER OR FACILITY THAT IS OWNED, OPERATED OR
36 GOVERNED BY A FEDERALLY RECOGNIZED INDIAN TRIBE IN THIS STATE.

37 (e) A TRIBAL REGIONAL BEHAVIORAL HEALTH AUTHORITY.

38 (f) AN URBAN INDIAN ORGANIZATION THAT IS CONTRACTED OR THAT
39 RECEIVES GRANTS TO PROVIDE SERVICES PURSUANT TO 25 UNITED STATES CODE
40 CHAPTER 18.

41 (g) A TRIBAL HEALTH FACILITY THAT IS OPERATED UNDER AN
42 INTERGOVERNMENTAL AGREEMENT BETWEEN TWO OR MORE INDIAN TRIBES IN THIS
43 STATE.}}

1 Sec. 2. Legislative findings

2 The legislature finds and determines that immediate implementation
3 of this act is necessary to strengthen program integrity and care
4 management for American Indian and Alaska native members of the Arizona
5 health care cost containment system, to protect those members from fraud
6 and abuse and to preserve the public peace, health and safety.

7 Sec. 3. Emergency

8 This act is an emergency measure that is necessary to preserve the
9 public peace, health or safety and is operative immediately as provided by
10 law.

11 Enroll and engross to conform

12 Amend title to conform

CARINE WERNER

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03/09/2026

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C: MH

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